



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : زينب فتحي عبد العظيم عبيد

العنوان : =

الحي / المدينة : الفشن

المحافظة : بني سويف

تاريخ تسجيل الإستمارة : 21-09-2026

رقم البطاقة : 27503092201728 رقم التليفون : 01146741414

العمر (تاريخ الميلاد) : 09-03-1975 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
 الاسم: د/ندى حسن الوظيفة: التوقيع:
 الاسم: د/نرمين رمضان الوظيفة: التوقيع:
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: زينب فتحي عبد العظيم عبد National ID: 27503092201728	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



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Name : Zeinab Fathy Abdelazim

Date : 10-1-2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Left breast, inferiorly located irregular enhancing parenchymal mass is seen measuring about 2.6 x 1.7 cm.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Bilateral basal pulmonary atelectatic bands.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is of average size with homogenous CT texture. Left hepatic lobe (segment II) focal lesion is seen measuring about 2.7 x 2.5 cm. It shows peripheral nodular enhancement at the portal phase with delayed filling in.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Left adnexal cyst is seen measuring about 5.3 x 3.6 cm
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic lymphadenopathy or collections.
- No gross suspicious lytic or sclerotic bone lesions.

OPINION:

- Left breast, inferiorly located malignant-looking parenchymal mass lesion.
- No pulmonary metastatic focal lesions.
- Left hepatic lobe (segment II) focal lesion with enhancement pattern consistent with hemangioma.
- Left adnexal cyst, for sonographic follow up.

Dr. Peter Nashaat, MSc

Dr. Amr Farouk, MD

Patient Name	: الأستاذة / زينب فتحى عبدالعظيم عبيد	Request No.	: 13004915
Patient No.	: 27503092201728	Request Date	: 22/05/2022
Path No.	: 1369/2022	Verification Date	: 04/06/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Lt breast cancer, Post NACT

Nature of the specimen: Lt WLE + SLNB

Gross Examination:

- Procedure: A wired WLE measured 6.5x3x2cm, skin ellipse measured 1.5x0.5cm
- Localization: Lt
- Tumor extent
 - o Number: An ill-defined wired greyish white rubbery oozing tumor bed site.
 - o Size: 2.5x1x1cm
 - o Skin involvement: Grossly unremarkable.
- Distance from margins: Least deep margin = 0.4cm
- Lymph nodes number and size: 2 nodules up to 0.7x0.4cm
- Frozen section: - Free margins. - Free (0/2) SLN.

Microscopic examination:

- Histological type: Invasive Ductal Carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1)
- Ductal carcinoma in situ: 4%
- Tumor extent(10%): 0.2 cm
- Lymph node involvement: Free (0/2) SLNs.
- Resection Margins:
 - o Free
- Histologic response to therapy: Pathological Partial Response
- Additional pathologic findings: mucoid changes (effect of therapy)

Diagnosis:

Lt breast cancer, Lt WLE + SLNB:

- Invasive Ductal Carcinoma GII, with 4% Ductal carcinoma in situ

Verified by
dr.menna atef

Printing Date : 08/06/2022 09:38:58

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Printing By : Ali mostafa

Patient Name : الأستاذ / زينب فتحى عبدالعظيم عبيد	Request No. : 13004915
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Age : 47 Y	Department : Out Patient
Referreing Doc :	Registered by : .marwa mohamed youss

Pathology Report

- Free (0/2) SLNs.
- Free Resection Margins
- Pathological Partial Response
- Stage: ypT1a, ypN0

Prof. Lubna Omar
Dr. Menna Atef

Verified by :
dr.menna atef

Printing Date : 08/06/2022 09:38:58

Page 2 of 2

Printing By : Ali mostafa

Patient Name: Zainab Fat'hy Abd Al Azeem	Date : 12/10/2023
Exam : ultrasound ABDOMEN& PELVIS	Age : 47 Year(s)
Medical ID: 3485/2021	Pat. ID: 11194

ABDOMINO-PELVIC SCAN OF YOUR PATIENT REVEALED:

LIVER

- **Enlarged** size measuring 17.8 cm showing **bright coarse** parenchymal echo-pattern, smooth outlines & normal hepatic veins.
- Average portal vein caliber with patent lumen.
- No focal lesions. No HBRD. No ascites.

GALL BLADDER

Average size & wall thickness with free sonolucent cavity.

Average caliber of C.B.D. with no detectable stones.

SPLEEN: average size (9.5 cm) with normal echogenicity & no focal lesions.

KIDNEYS

Normal size, parenchymal thickness and echogenicity.

No stones or backpressure changes could be noted.

URINARY BLADDER: empty at time of examination.

Uterus: AVF, average size with homogeneous myometrium

Midline pelvic adnexal well-defined multiloculated cystic lesion measuring 7.3 x 7.5 cm with no evidence of internal vascularity, thin wall and thin septations with posterior acoustic enhancement

OPINION:

Chronic liver disease as described for lab correlation

Midline pelvic multilocular cystic lesion as described (O-RADS 3: low risk of malignancy)

MRI pelvis with contrast is recommended for better tissue characterization

Dr/Lamiaa Elsharkawy, M.D.

Signature Dr. Lamyaa Nagah

12/10/2023 1:32 PM



Patient Name: Zainab Fat'hy Abd Al Azeem	Date : 19/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 47 Year(s)
Medical ID: 3485/2021	

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had It CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring.
- Complementary ultrasound showed left breast LOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left and right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly ,

Dr/Asmaa Zeinhom, Msc

proved

Patient Name: Zienb Fathy Abdel-Azlem.

Date: 10/10/2021

^{99m}Tc - ^{99m}Tc -MDP BONE SCAN

Desired Information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- Degenerative changes are noticed over dorso-lumbar vertebrae.
- No definite hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions.

Thanks for reference

Nuclear Medicine Unit

DR. Sherin Mahdy, MD

Patient's Name:	Zainab Fathy Abdel Azim Obeid
Date:	2026-08-18
Referred by:	Prof.Dr. no doctor
Hospital:	Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.

ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

I- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. Left hepatic lobe focal lesion seen at segment II measuring 20 x 17 mm showed homogenous enhancement
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ Left hepatic lobe focal lesion ?? hemangioma for triphasic study.

Dr.Asseer Ahmed

Patient's Name: Zainab Fathy Abdel Azim Obeid

Date: 2026-08-18

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Status post left CBS , routine follow up study.

Findings:

Left breast.

- Scattered fibroglandular densities. (ACR b)
- Mild Thickened skin and coarsen trabeculae.
- Center of lower quadrants shows architectrual distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- **Ultrasound** demonstrated at Center of lower quadrants area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin and coarsen trabeculae.
- Nonspecific axillary lymph node with preserved fatty hilum Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Right breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion:-

- *Left breast (BIRADS 2): Post-Therapeutic sequelae.*
- *Right breast (BIRADS 1): Normal study*

Recommendation:-

- *For annual follow up.*

Best Regards,

DR. Marwa M.Hassan